

Women’s Health Requirements Discovery Pack

Clinical Validation

Please mark whether you agree with each interpretation of the attached guideline and amend anything that does not match local practice.

Prepared for requirement discussion.

Service	Category or scenario	Source document	Our understanding of the rule or decision point	What information does the grader need to make this decision?	Expected outcome or priority	Agree as written? (Yes / No / Amend / Not used locally)	If not please amend	Needed in phase 1? (Yes / No / Later)	Clinical owner	Notes
COLPOSCOPY	Note on colposcopy rule extraction	Detailed extraction prepared from scanned colposcopy guide	The rows below reflect a detailed transcription and normalization of the booking-priority guide. Please confirm whether these interpretations match local practice and amend any wording or timing that differs.	HPV result, HPV group, cytology group, immune status, referral context, repeat or prior history				Yes		
COLPOSCOPY	Primary screening: HPV 16/18 with cytology reporting suspicion or definite cancer	Detailed colposcopy extraction	HPV 16/18 with cytology reporting suspicion or definite cancer -> 10 days.	HPV genotype and cytology result	10 days			Yes		
COLPOSCOPY	Primary screening: HPV 16/18 with or without cytology, any cytology excluding cancer	Detailed colposcopy extraction	HPV 16/18 with or without cytology, any cytology result excluding cancer -> 30 days.	HPV genotype and cytology result or absence of cytology	30 days			Yes		
COLPOSCOPY	Primary screening: HPV other with cytology reporting suspicion or definite cancer	Detailed colposcopy extraction	HPV other with cytology reporting suspicion or definite cancer -> 10 days.	HPV group and cytology result	10 days			Yes		
COLPOSCOPY	Primary screening: HPV other with ASC-H, HSIL, AIS, or glandular cytology	Detailed colposcopy extraction	HPV other with cytology reporting ASC-H, HSIL, AIS, or glandular -> 30 days.	HPV group and cytology result	30 days			Yes		
COLPOSCOPY	Primary screening: second HPV other with no cytology or normal, ASC-US, LSIL >50	Detailed colposcopy extraction	Second HPV other with no cytology or cytology normal, ASC-US, LSIL >50 -> 3 months.	Repeat count, HPV group, cytology, age split where used	3 months			Yes		
COLPOSCOPY	Primary screening: third HPV other with no cytology or normal, ASC-US, LSIL <50	Detailed colposcopy extraction	Third HPV other with no cytology or cytology normal, ASC-US, LSIL <50 -> 6 months.	Repeat count, HPV group, cytology, age split where used	6 months			Yes		
COLPOSCOPY	Immune-deficient direct referral: no cytology	Detailed colposcopy extraction	Immune deficient with no cytology -> 3 months.	Immune status, cytology availability	3 months			Yes		
COLPOSCOPY	Immune-deficient direct referral: cytology normal, ASC-US, or LSIL	Detailed colposcopy extraction	Immune deficient with cytology normal, ASC-US, LSIL -> 6 months.	Immune status and cytology result	6 months			Yes		
COLPOSCOPY	Abnormal appearance: clinical suspicion or definite cancer	Detailed colposcopy extraction	Abnormal cervical appearance with clinical suspicion or definite cancer -> 10 days.	Cervical appearance and clinical concern	10 days			Yes		
COLPOSCOPY	Abnormal appearance: normal sample history including current sample	Detailed colposcopy extraction	Abnormal cervical appearance with normal sample history, including current sample -> 3 months.	Appearance plus screening history	3 months			Yes		
COLPOSCOPY	Other reasons: HPV not detected and suspicion of cancer	Detailed colposcopy extraction	HPV not detected and suspicion of cancer, with or without cytology -> 10 days.	HPV result, cancer suspicion, cytology if present	10 days			Yes		
COLPOSCOPY	Other reasons: HPV not detected with ASC-H, HSIL, AIS, or glandular cytology	Detailed colposcopy extraction	HPV not detected with cytology reporting ASC-H, HSIL, AIS, or glandular -> 30 days.	HPV result and cytology result	30 days			Yes		
COLPOSCOPY	Other reasons: other clinical assessment (ambiguous in guide)	Detailed colposcopy extraction	The guide lists 'Other reason: Other clinical assessment' at 30 days, 3 months, and 6 months, but does not show the condition that differentiates these three timings. Please clarify the actual rule.	Reason for referral and local clinical rule	Please confirm 30 days / 3 months / 6 months criteria			Yes		
COLPOSCOPY	Positive test of cure re-referral: HPV 16/18 or HPV other with suspicion or definite cancer	Detailed colposcopy extraction	HPV 16/18 or HPV other and cytology reporting suspicion or definite cancer -> 10 days.	Test-of-cure context, HPV group, cytology result	10 days			Yes		
COLPOSCOPY	Positive test of cure re-referral: HPV 16/18 or HPV other with ASC-H, HSIL, AIS, or glandular	Detailed colposcopy extraction	HPV 16/18 or HPV other and cytology reporting ASC-H, HSIL, AIS, or glandular -> 30 days.	Test-of-cure context, HPV group, cytology result	30 days			Yes		

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COLPOSCOPY	Positive test of cure re-referral: HPV 16/18 or HPV other with no cytology	Detailed colposcopy extraction	HPV 16/18 or HPV other and no cytology -> 3 months.	Test-of-cure context, HPV group, cytology availability	3 months			Yes		
COLPOSCOPY	Positive test of cure re-referral: HPV 16/18 or HPV other with normal or ASCUS/LSIL cytology	Detailed colposcopy extraction	HPV 16/18 or HPV other and cytology normal or ASCUS/LSIL -> 3 months.	Test-of-cure context, HPV group, cytology result	3 months			Yes		
COLPOSCOPY	Positive test of cure re-referral: HPV not detected with ASCUS/LSIL x2	Detailed colposcopy extraction	HPV not detected and cytology ASCUS/LSIL x2 -> 6 months.	Test-of-cure context, HPV result, repeat count, cytology result	6 months			Yes		
COLPOSCOPY	Previous normal colposcopy re-referral: HPV 16/18 or HPV other with suspicion or definite cancer	Detailed colposcopy extraction	HPV 16/18 or HPV other and cytology reporting suspicion or definite cancer -> 10 days.	Prior normal colposcopy context, HPV group, cytology result	10 days			Yes		
COLPOSCOPY	Previous normal colposcopy re-referral: HPV 16/18 or HPV other with ASC-H, HSIL, AIS, or glandular	Detailed colposcopy extraction	HPV 16/18 or HPV other and cytology reporting ASC-H, HSIL, AIS, or glandular -> 30 days.	Prior normal colposcopy context, HPV group, cytology result	30 days			Yes		
COLPOSCOPY	Previous normal colposcopy re-referral: HPV 16/18 with no cytology	Detailed colposcopy extraction	HPV 16/18 and no cytology -> 6 months.	Prior normal colposcopy context, HPV group, cytology availability	6 months			Yes		
COLPOSCOPY	Previous normal colposcopy re-referral: HPV 16/18 with normal or ASCUS/LSIL cytology	Detailed colposcopy extraction	HPV 16/18 and cytology normal or ASCUS/LSIL -> 6 months.	Prior normal colposcopy context, HPV group, cytology result	6 months			Yes		
COLPOSCOPY	Previous normal colposcopy re-referral: immune-deficient HPV other with no cytology or normal / ASCUS / LSIL	Detailed colposcopy extraction	Immune deficient HPV other; no cytology or cytology normal, ASCUS/LSIL -> 6 months.	Prior normal colposcopy context, immune status, HPV group, cytology	6 months			Yes		
COLPOSCOPY	Previous normal colposcopy re-referral: HPV other with normal or ASCUS/LSIL x2	Detailed colposcopy extraction	HPV other and cytology normal or ASCUS/LSIL x2 -> 6 months.	Prior normal colposcopy context, HPV group, repeat count, cytology result	6 months			Yes		
COLPOSCOPY	Previous normal colposcopy re-referral: HPV other with no cytology x2	Detailed colposcopy extraction	HPV other and no cytology x2 -> 6 months.	Prior normal colposcopy context, HPV group, repeat count, cytology availability	6 months			Yes		
COLPOSCOPY	Previous LSIL histology re-referral: HPV 16/18 or HPV other with suspicion or definite cancer	Detailed colposcopy extraction	HPV 16/18 or HPV other with cytology reporting suspicion or definite cancer -> 10 days.	Previous LSIL histology context, HPV group, cytology result	10 days			Yes		
COLPOSCOPY	Previous LSIL histology re-referral: HPV 16/18 or HPV other with ASC-H, HSIL, AIS, or glandular	Detailed colposcopy extraction	HPV 16/18 or HPV other and cytology reporting ASC-H, HSIL, AIS, or glandular -> 30 days.	Previous LSIL histology context, HPV group, cytology result	30 days			Yes		
COLPOSCOPY	Previous LSIL histology re-referral: HPV 16/18 with no cytology	Detailed colposcopy extraction	HPV 16/18 and no cytology -> 3 months.	Previous LSIL histology context, HPV group, cytology availability	3 months			Yes		
COLPOSCOPY	Previous LSIL histology re-referral: HPV 16/18 with normal or ASCUS/LSIL cytology	Detailed colposcopy extraction	HPV 16/18 and cytology normal or ASCUS/LSIL -> 6 months.	Previous LSIL histology context, HPV group, cytology result	6 months			Yes		
COLPOSCOPY	Previous LSIL histology re-referral: immune-deficient HPV other with no cytology or normal / ASCUS / LSIL	Detailed colposcopy extraction	Immune deficient HPV other; no cytology or cytology normal, ASCUS/LSIL -> 6 months.	Previous LSIL histology context, immune status, HPV group, cytology	6 months			Yes		
COLPOSCOPY	Previous LSIL histology re-referral: HPV other with normal or ASCUS/LSIL x2	Detailed colposcopy extraction	HPV other and cytology normal or ASCUS/LSIL x2 -> 6 months.	Previous LSIL histology context, HPV group, repeat count, cytology result	6 months			Yes		
COLPOSCOPY	Previous LSIL histology re-referral: HPV other with no cytology x2	Detailed colposcopy extraction	HPV other and no cytology x2 -> 6 months.	Previous LSIL histology context, HPV group, repeat count, cytology availability	6 months			Yes		
COLPOSCOPY	Faster Cancer Treatment (FCT) pathway	Grading template	The grading template includes an FCT field with three options: Confirmed cancer at grading, High suspicion of cancer at grading, No/Low suspicion of cancer at grading. Please confirm when each applies.	Clinical findings, histology, imaging if relevant	FCT classification -- please confirm criteria			Yes		
COLPOSCOPY	Ovestin prescribing	Grading template	The grading template includes Ovestin prescribing options for patients who are amenorrhoeic, >50yo, or post-menopausal. Please confirm the rules for when each option applies.	Patient age, menopausal status, breastfeeding, Depo use, hormone contraindication	Ovestin instruction to referrer -- please confirm rules			Yes		

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COLPOSCOPY	NCSR notes	Grading template	The grading template includes NCSR note options (published to Whaihua/Register, not applicable, internal referral note). Please confirm when each is used.	NCSR registration status, referral type	NCSR note type -- please confirm			Yes		
COLPOSCOPY	Grading template fields -- please confirm which are required	Grading template image	The current grading template appears to include: Status Reason, Priority, FCT, Clinic, HPV test, HPV Type, Cytology sample, Referrer's reason for referral, Genotype, Investigations, Category, Assessment of referral, Booking priority, Type, To be seen by SMO only, Ovestin, NCSR notes, Triage Notes (internal), Notes. Please mark which fields are required, which are optional, and which ones drive the grading decision.	All of the above	Confirmed field list			Yes		
GYNAECOLOGY	AUB -- no ultrasound within 12 months	Gynaecology guideline PDF	If no pelvic USS within 12 months: reject and send back to GP requesting re-referral with scan.	Referral content, USS availability	Reject -- request re-referral with USS			Yes		
GYNAECOLOGY	AUB -- ET >15mm or no recent endometrial sample, with or without adenomyosis	Gynaecology guideline PDF	If USS shows endometrial thickness >15mm and no pipelle or endometrial sample in last 6 months, with or without adenomyosis: P2 Menorrhagia.	USS findings, endometrial thickness, biopsy history	P2 Menorrhagia			Yes		
GYNAECOLOGY	AUB -- USS suggestive of polyp	Gynaecology guideline PDF	If USS suggestive of polyp with or without dysmenorrhoea: P3 Hysteroscopy.	USS findings, symptoms	P3 Hysteroscopy			Yes		
GYNAECOLOGY	AUB -- ET <15mm, normal USS, but ongoing bleeding and tried medical management	Gynaecology guideline PDF	If ET <15mm and otherwise normal but intermenstrual or continuous bleeding >3 months and patient has tried medical management (Mirena, Provera, TXA, COC, POP), with or without dysmenorrhoea or adenomyosis: P3 Menorrhagia. If not tried medical management, suggest trying first.	USS, bleeding duration, medications tried	P3 Menorrhagia (or suggest medical management first)			Yes		
GYNAECOLOGY	AUB -- post-coital bleeding with normal smear (premenopausal)	Gynaecology guideline PDF	Post-coital bleeding with normal smear in premenopausal patient: does not need to be seen (Northern Region Agreement).	Smear result, menopausal status	Does not need to be seen			Yes		
GYNAECOLOGY	AUB -- small fibroid (<3cm) found incidentally on USS	Gynaecology guideline PDF	If USS shows fibroid <3cm with no other abnormalities and normal ET: decline with advice that fibroid is incidental, try medical management. P3 Gynae if already tried medical management.	USS, fibroid size, ET, management tried	Decline (or P3 if tried medical management)			Yes		
GYNAECOLOGY	AUB -- abnormal cervix with normal smear (SMO grading)	Gynaecology guideline PDF	If abnormal cervix with normal smear: P2 Gynae. SMO grading required.	Cervical appearance, smear result	P2 Gynae (SMO only)			Yes		
GYNAECOLOGY	PMB -- no USS within 6 months	Gynaecology guideline PDF	No USS within 6 months: reject and ask to re-refer with USS (suggest POAC if unable to afford). Exception: clear historic abnormality with delay to review.	USS availability, clinical history	Reject -- request USS			Yes		
GYNAECOLOGY	PMB -- single episode, USS <4mm	Gynaecology guideline PDF	Single episode PMB with USS ET <4mm polyp: suggest community pipelle if able, ovestin cream, and refer if recurrent PMB.	PMB episodes, USS findings, ET	Suggest community pipelle and ovestin, re-refer if recurrent			Yes		
GYNAECOLOGY	PMB -- one or more episodes, USS ET >=5mm	Gynaecology guideline PDF	One or more episodes of PMB with USS ET >=5mm: HSC P1, Hysteroscopy Rapid Access Clinic.	PMB episodes, USS findings, ET	HSC P1 -- RAC clinic			Yes		
GYNAECOLOGY	PMB -- multiple episodes but ET <5mm	Gynaecology guideline PDF	Multiple episodes of PMB but ET <5mm: HSC P1, RAC clinic.	PMB episodes, USS findings, ET	HSC P1 -- RAC clinic			Yes		
GYNAECOLOGY	PMB -- USS shows other abnormalities (SMO grading)	Gynaecology guideline PDF	If USS shows any other abnormalities: SMO-only grading, clinical judgement.	USS findings	SMO clinical judgement			Yes		

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GYNAECOLOG	Fibroids	Gynaecology guideline PDF	Fibroids >3cm with mass symptoms (urinary, bowel, pain) and/or hydronephrosis: P2 Gynae. Fibroids <3cm with mass symptoms: P3 Gynae (note to GP to explore other causes). Fibroids <3cm with no mass symptoms or AUB: decline, no follow-up scan needed.	Fibroid size, symptoms, USS findings	P2 or P3 or Decline depending on size and symptoms			Yes		
GYNAECOLOG	Ovarian masses and cysts	Gynaecology guideline PDF	Probable benign ovarian cysts (simple or likely dermoid): Gynae P2. Complex adnexal masses that appear suspicious: SMO-only grading, order CT if >40, tumour markers all ages, HSC P1.	Mass characteristics, imaging, tumour markers, age	P2 or HSC P1 (SMO only for suspicious)			Yes		
GYNAECOLOG	HSC P2 -- semi-urgent (cross-cutting category)	Gynaecology guideline PDF	HSC P2 is an SMO-only category for referrals with concerning features that do not qualify for HSC P1 but should not wait the standard P2 timeframe. Examples: borderline ovarian cyst, recurrent PMB with normal histology, premenopausal with significantly increased ET. Tracked by Gynaecology Nurse Coordinator.	Clinical judgement, imaging, histology	P2-HSC (SMO only, tracked by nurse coordinator)			Yes		
GYNAECOLOG	Pelvic pain	Gynaecology guideline PDF	No USS: reject unless extensive endometriosis history. Pelvic pain >3 months with normal USS: P3. Abnormal USS with endometriomas >=5cm: P2. DIE on imaging: P2. Chronic pelvic pain fully investigated and gynae cause treated: decline, refer to Chronic Pain Service.	USS, pain duration, prior treatment, imaging	P2 or P3 or Reject or Decline depending on findings			Yes		
GYNAECOLOG	Urogynaecology	Gynaecology guideline PDF	Multiple sub-pathways including mesh problems (P2, SMO), procidentia (P2), prolapse without procidentia (P3), prolapse with urinary retention (P2), physiotherapy referral (P2), pessary, incontinence (P3 for stress, reject for urge unless tried medications, reject for asymptomatic prolapse), previous TVT/sling (P2, SMO). Urogynae patient with PMB follows PMB grading.	Presenting issue, examination findings, prior treatment, mesh history	Multiple outcomes depending on sub-category			Yes		
GYNAECOLOG	Fertility	Gynaecology guideline PDF	Decline and advise GP to refer directly to Northern Region Fertility Service (NRFS).	Referral reason	Decline -- redirect to NRFS			Yes		
GYNAECOLOG	PCOS	Gynaecology guideline PDF	Virtual only. Women with PCOS not seen face-to-face. Refer GP to PCOS guidelines on Health Pathways. Refer for clinician advice if diagnosis unclear. SMO if fertility component.	Symptoms, fertility concern, diagnosis clarity	Virtual or redirect			Yes		
GYNAECOLOG	Cervical polyp	Gynaecology guideline PDF	Asymptomatic, <=2cm with normal smear: reject, GP to monitor. >=2cm with normal smear and symptoms (post-coital or intermenstrual bleeding): P3 Gynaecology.	Polyp size, symptoms, smear result	Reject or P3 depending on size and symptoms			Yes		
GYNAECOLOG	Paediatric gynaecology and structural anomalies	Gynaecology guideline PDF	SMO grading. Except menstrual disorders -- may be reviewed at Counties Manukau with named consultants. Young women with raised tumour markers and complex cysts go to FCT team unless clearly endometrioma. Otherwise transfer to Paediatric Gynaecology / Female Multi Clinic Te Toka Tumai.	Age, presenting issue, imaging, tumour markers	SMO grading, possible transfer to paediatric service			Yes		
GYNAECOLOG	Pelvic tear	Gynaecology guideline PDF	3A: refer to WH physio. 3B / 3C / 4th degree: P3.	Tear grade, symptoms	Physio referral or P3			Yes		
GYNAECOLOG	Tubal ligation	Gynaecology guideline PDF	Tubal ligation counselling (MSC): P3.	Referral reason	P3			Yes		

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GYNAECOL GY	Reject, return, or request-more-information rules	Gynaecology guideline PDF	Several categories require minimum evidence before grading (e.g. USS for AUB, PMB, pelvic pain). Referrals without required evidence should be returned to GP with a specific request. The service also has decline outcomes for conditions that do not warrant specialist review.	Minimum investigations, referral completeness	Reject, return to GP, or decline with advice			Yes		
GYNAECOL GY	Escalation -- re-grading patients already on the waitlist	Gynaecology guideline PDF	Patients already on a waitlist may be re-referred with updated clinical information. These must be re-graded and the booking team notified if priority changes.	Updated referral, prior grading, new clinical information	Re-grading and potential priority change			Yes		